INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Phesgo maintenance

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
21 days
See the active treatment phase

Treatment stages
1
Explained in the treatment reference

Regimen source
NCCP 00785
Version 3

> Phesgo on Day 1 every 21 days after the chemotherapy component is complete; continue while beneficial and tolerated. This is active metastatic maintenance, not a fixed one-year adjuvant course.

Treatment stages

01 **Phesgo maintenance**
21-day cycle

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

Phesgo maintenance
National Cancer Control Programme (NCCP) 00785

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Contact oncology promptly for new breathlessness, swelling, severe diarrhoea, inability to drink or sudden illness. Tell staff immediately about rash, chest tightness or breathing difficulty during the injection.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

Two antibodies in one injection

Control the cancer and symptoms, with repeated review of benefit, tolerance and your priorities.

HER2-TARGETED ANTIBODY

Pertuzumab

Part of the combined Phesgo injection



Blocks HER2 pairing with other receptors

Pertuzumab binds a different area of HER2 from trastuzumab. It limits receptor pairing, so the two antibodies act together at different points on the same growth pathway.

HER2 pairing → Pertuzumab blocks pairing → Growth signals reduced

What to watch for

Keep echo appointments. Report diarrhoea, new breathlessness, swelling, palpitations or an injection reaction.

HER2-TARGETED ANTIBODY

Trastuzumab

Part of the combined Phesgo injection



Targets HER2 growth signals

Trastuzumab binds to the HER2 protein on cells and helps reduce HER2-driven growth and recruit immune activity. HER2 testing identifies the cancers for which this approach is used.

HER2 on cell surface → Antibody binds → Growth signalling reduced

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Why this treatment?

Your chemotherapy phase has finished. The two medicines in Phesgo continue to target HER2-positive cancer while your team reviews cancer control and side effects. Maintenance is still active treatment.

Why the HER2 medicines continue

Why does this phase feel different?

Phesgo contains pertuzumab and trastuzumab in one injection under the skin. It contains no conventional chemotherapy. The plan depends on earlier treatment, response, heart health and your priorities. This advanced-cancer pathway has no fixed one-year stopping point.

Your own decision

Ask which cancer features, earlier treatments and health factors led to this recommendation, what alternatives were considered, and when the plan will be reviewed.

The aim and the whole plan

Who this is for

Completed the chemotherapy component of dual HER2 treatment for metastatic or locally recurrent unresectable disease.

Schedule

Phesgo on Day 1 every 21 days after the chemotherapy component is complete; continue while beneficial and tolerated. This is active metastatic maintenance, not a fixed one-year adjuvant course.

After this phase

After chemotherapy stops, use the agreed Phesgo maintenance plan (NCCP 00785 when eligible). Keep cardiac and response checks; chemotherapy steroids, antiemetics and growth factors do not automatically continue.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

Phesgo maintenance
NCCP 00785

How the treatment phases connect

Phesgo maintenance

21-day view



This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

21-DAY CYCLE

Phesgo maintenance

Review while benefiting

Phesgo on Day 1 every 21 days after the chemotherapy component is complete; continue while beneficial and tolerated. This is active metastatic maintenance, not a fixed one-year adjuvant course.

BASELINE → TREATMENT → FOLLOW-UP

When checks happen

Columns show order, not equal durations. The week-12 echo is relative to first HER2 treatment, not the first Phesgo injection after a switch. Booking and result review remain the treating team's responsibility.

Check	Before starting	Cycles 1–4	Around cycle 5 / week 12	Continuing treatment	Transition / follow-up
Blood tests	full blood count (FBC), renal and liver profile	FBC, renal and liver profile before every cycle.	Continue pre-cycle bloods	FBC, renal and liver profile before every cycle.	Review tests for the next active medicine
Echo / multigated acquisition (heart pumping) scan (MUGA)	Before the first HER2 treatment	Earlier if symptoms or concern	Echo due at about 12 weeks (cycle 5 if on time)	Every 12 weeks; 4–8 weeks if impairment	Keep the clock on maintenance
electrocardiogram (ECG) / cardiac markers	ESC: baseline ECG and cardiovascular risk review; baseline troponin/BNP if they will be used for monitoring	Repeat when symptoms, risk or the cardiac plan indicate	ECG does not replace echo	Markers are risk-based, not automatic tumour-marker tests	Follow up abnormal results

Check	Before starting	Cycles 1–4	Around cycle 5 / week 12	Continuing treatment	Transition / follow-up
Scans / cancer markers	Baseline imaging of known disease	Clinical response; earlier imaging for concern	Response review generally every 2–4 months	Repeat appropriate imaging; markers only as adjuncts	Review benefit and next-line plan
dual-energy X-ray absorptiometry (bone density) (DEXA) / bone health	Assess endocrine indication and fracture risk	No routine cycle-by-cycle DEXA	Plan baseline DEXA if aromatase inhibitor (AI) / ovarian suppression indicated	Repeat according to risk, results and endocrine therapy	Continue the agreed bone-health plan

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00785 v3 · official regimen · ESMO 2024 early breast cancer guideline · treatment and follow-up · ESC 2022 cardio-oncology guideline · cardiac surveillance and risk · NICE NG101 · bone health and follow-up · ESMO metastatic breast cancer guideline · 2026 · EMA Phesgo product information

THE WHOLE TREATMENT JOURNEY

Phesgo maintenance: the whole pathway

HER2-positive metastatic / unresectable locally recurrent breast cancer · maintenance after chemotherapy

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Baseline / previous treatment	Phesgo maintenance	Response / phase review	Continuing Phesgo	Change / follow-up
Cancer medicines NCCP 00785	Confirm HER2 status, indication and baseline checks	Phesgo every 21 days	Review benefit and tolerance	Phesgo alone while beneficial and tolerated	Review outcomes and next plan
Surgery METASTATIC	Only if individually indicated	Coordinate with systemic treatment	multidisciplinary team (MDT) review	No routine surgical phase	Site-specific plan
Radiotherapy METASTATIC	Assess symptom / site need	Timing and pauses agreed by both teams	Reassess need	No assumed routine concurrent schedule	Site-specific review

Care / check	Baseline / previous treatment	Phesgo maintenance	Response / phase review	Continuing Phesgo	Change / follow-up
Blood tests NCCP 00785	Baseline FBC, renal / liver	FBC, renal and liver profile before every cycle.	Check before changing phase	FBC, renal and liver profile before every cycle.	Active medicines / abnormal results determine follow-up
Echo / heart tests NCCP 00785 · ESC · SMPC	Baseline echo / MUGA; ECG risk review	Symptoms can prompt earlier assessment	12 weeks from first HER2 dose; around cycle 5	Continue every 12 weeks including maintenance	Agree follow-up when treatment ends
Imaging / markers METASTATIC · MARKERS	Baseline disease assessment	Review clinical response	Generally every 2–4 months	Adapt to symptoms and known sites	Discuss results and next steps
Endocrine / DEXA BONE · EARLY	Check hormone receptors and fracture risk	Discuss the later plan	Baseline DEXA when indicated	Endocrine therapy may overlap HER2 / radiotherapy (RT) if agreed	Bone and endocrine follow-up

What each check is for

1

Blood tests

FBC, renal and liver profile before every cycle.

Check treatment safety.

Ask / act: The treating team records the booking owner, due window, result reviewer and action if overdue.

2

Echo / MUGA

Echo or MUGA before starting and every 12 weeks during treatment, including maintenance. At an uninterrupted 21-day schedule, the first 12-week check is around cycle 5; count elapsed time from the first HER2 dose. A delay or route/phase change does not restart the clock. Cardiac concern may require 4–8-week checks or urgent earlier assessment.

Monitor heart pumping function.

Ask / act: The treating team records the booking owner, due window, result reviewer and action if overdue.

3

Cancer imaging

Baseline imaging records the disease sites. ESMO recommends response review generally every 2–4 months, adjusted for symptoms, disease pace, sites and treatment. computed tomography (CT) chest/abdomen/pelvis is used when appropriate; the team selects the actual scan and review date. Tumour markers may supplement assessment, not replace scans or determine a treatment change alone.

Assess disease response or surveillance.

Ask / act: The treating team records the booking owner, due window, result reviewer and action if overdue.

4

Bone health

If hormone-receptor-positive disease leads to an aromatase inhibitor or ovarian suppression, assess fracture risk and baseline DEXA when indicated. Repeat according to results, risk and the endocrine plan; DEXA is not a scan at each Phesgo cycle.

Reduce fracture risk when endocrine therapy affects bone.

Ask / act: The treating team records the booking owner, due window, result reviewer and action if overdue.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Your practical treatment plan

Phesgo is one combined injection of pertuzumab and trastuzumab under the skin of the thigh. It is not two separate injections and is never given into a vein.

STEP 1

Before the visit

Confirm blood results, cardiac checks, active phase, previous dual HER2 dose and the consent discussion.

STEP 2

The thigh injection

Loading/reloading 1200 mg/600 mg over 8 minutes with 30 minutes observation; maintenance 600 mg/600 mg over 5 minutes with 15 minutes observation. Alternate thighs; avoid sore, bruised or damaged skin.

STEP 3

No intravenous (IV) line for Phesgo itself

Phesgo is given under the skin and does not itself require a port, cannula or take-home pump. Other treatment may still require access.

STEP 4

Leaving the unit

Confirm supportive medicines, next visits and due tests. Injection time is shorter than the whole appointment; preparation, assessment and observation still take time.

Switching / a missed appointment

The team checks the last dual HER2 dose, including IV treatment: a gap of less than 6 weeks uses the maintenance dose; 6 weeks or longer needs reloading. A first Phesgo visit after IV treatment does not automatically require a loading dose.

An existing port

Ask the line team whether the port is still needed and how it will be cared for or removed. Removal is a separate procedure; do not adjust the device yourself.

Scans, heart checks and bloods

Agree who books, reviews and acts on each result.

Blood tests

FBC, renal and liver profile before every cycle.

Heart ultrasound

Echo or MUGA before starting and every 12 weeks during treatment, including maintenance. At an uninterrupted 21-day schedule, the first 12-week check is around cycle 5; count elapsed time from the first HER2 dose. A delay or route/phase change does not restart the clock. Cardiac concern may require 4–8-week checks or urgent earlier assessment.

Longer-term cardiac plan

NCCP continues 12-week checks. ESC permits six-monthly echo only after the first year in selected asymptomatic low/moderate-risk metastatic patients with normal assessments, following an agreed specialist plan; moving to maintenance alone does not qualify.

Cancer imaging and markers

Baseline imaging records the disease sites. ESMO recommends response review generally every 2–4 months, adjusted for symptoms, disease pace, sites and treatment. CT chest/abdomen/pelvis is used when appropriate; the team selects the actual scan and review date. Tumour markers may supplement assessment, not replace scans or determine a treatment change alone.

Surgery, radiotherapy and hormone treatment

Other treatment depends on your cancer and personal plan.

How surgery and radiation fit

Radiotherapy or surgery may be used for selected sites or symptoms. The teams decide timing and any pause in systemic therapy; there is no routine concurrent radiotherapy schedule in this NCCP regimen.

Endocrine treatment and bone health

If hormone-receptor-positive disease leads to an aromatase inhibitor or ovarian suppression, assess fracture risk and baseline DEXA when indicated. Repeat according to results, risk and the endocrine plan; DEXA is not a scan at each Phesgo cycle.

Receptor and genomic tests

Hormone-receptor status can affect endocrine treatment. Oncotype-type chemotherapy triage is not applied to these HER2-positive regimens; treatment selection requires the appropriate indication-specific MDT assessment.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Phesgo maintenance
NCCP 00785

Phesgo maintenance

Day 1 → Day 21: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Phesgo · Day 1	 **Day 1** One subcutaneous (SC) injection
Diarrhoea	 **Days 1–21** Report early; keep drinking
Injection reactions	 **Days 1–21** Report during or after visits
Heart / breathing symptoms	 **Days 1–21** Report symptoms on any day

No dates to enter: Use this picture to understand the sequence. Your team provides your personal appointments.

Changing phase: After chemotherapy stops, use the agreed Phesgo maintenance plan (NCCP 00785 when eligible). Keep cardiac and response checks; chemotherapy steroids, antiemetics and growth factors do not automatically continue.

Echo clock: Echo or MUGA before starting and every 12 weeks during treatment, including maintenance. At an uninterrupted 21-day schedule, the first 12-week check is around cycle 5; count elapsed time from the first HER2 dose. A delay or route/phase change does not restart the clock. Cardiac concern may require 4–8-week checks or urgent earlier assessment.

Why these bands stay broad: Treatment days show the planned pattern. Symptom bands are broad possibilities, not measured risk curves or a prediction of when you will feel unwell. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00785 v3 · official regimen ↗

There are no guaranteed "safe days". Serious symptoms need assessment on any day, even if the chart shows a usual recovery period.

Between-treatment notes

Day 1–21: A reference cycle, without dates to enter

Phesgo on Day 1 every 21 days after the chemotherapy component is complete; continue while beneficial and tolerated. This is active metastatic maintenance, not a fixed one-year adjuvant course. Your team books actual appointments and reviews results. Report urgent symptoms on any day.

Support around the medicines

1 Antiemetic plan

Phesgo alone has minimal emetic risk; routine prophylactic antiemetics are not populated.

2 Premedicines

No routine Phesgo premedication unless indicated by prior reaction or the individual plan.

3 Appointments and results

Echo before treatment and every 12 weeks (around cycle 5 if on time), including maintenance. Bloods before each cycle. Ask who books and reviews each test; report a missing appointment.

The injection and your visit



Injection and observation

Questions worth asking

Will I need CT scans, markers or DEXA?

Baseline imaging records the disease sites. ESMO recommends response review generally every 2–4 months, adjusted for symptoms, disease pace, sites and treatment. CT chest/abdomen/pelvis is used when appropriate; the team selects the actual scan and review date. Tumour markers may supplement assessment, not replace scans or determine a treatment change alone. If hormone-receptor-positive disease leads to an aromatase inhibitor or ovarian suppression, assess fracture risk and baseline DEXA when indicated. Repeat according to results, risk and the endocrine plan; DEXA is not a scan at each Phesgo cycle.

Why was this treatment chosen for me?

Your chemotherapy phase has finished. The two medicines in Phesgo continue to target HER2-positive cancer while your team reviews cancer control and side effects. Maintenance is still active treatment. Phesgo contains pertuzumab and trastuzumab in one injection under the skin. It contains no conventional chemotherapy. The plan depends on earlier treatment, response, heart health and your priorities. This advanced-cancer pathway has no fixed one-year stopping point.

Try explaining the plan back: Which medicines are active now, when is your next echo or scan due, and who will review it?

Pertuzumab

- Diarrhoea and dehydration
- Injection / allergic reaction
- Heart pumping-function reduction
- Rash and fatigue

Trastuzumab

- Heart pumping-function reduction / heart failure
- Injection or allergic reaction
- Diarrhoea and fatigue
- Rare serious lung effects

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Phesgo-only maintenance

Active medicines: Pertuzumab + trastuzumab (Phesgo)

Emetogenic risk: Minimal for Phesgo / trastuzumab alone

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- No routine prophylactic antiemetic or cytotoxic premedication is carried over. Previous reactions or symptoms need an individual plan.
- Phesgo premedication is not usually required; consider paracetamol/antihistamine for the individual reaction history.
- After chemotherapy stops, use the agreed Phesgo maintenance plan (NCCP 00785 when eligible). Keep cardiac and response checks; chemotherapy steroids, antiemetics and growth factors do not automatically continue.
- Continue cardiac monitoring; maintenance does not restart the echo clock.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Phesgo maintenance

Phesgo on Day 1 every 21 days after the chemotherapy component is complete; continue while beneficial and tolerated. This is active metastatic maintenance, not a fixed one-year adjuvant course.

Source reconciled; independent clinical, pharmacy and patient/PPI review pending. Selected effects; not a complete medicine leaflet.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Heart / breathing

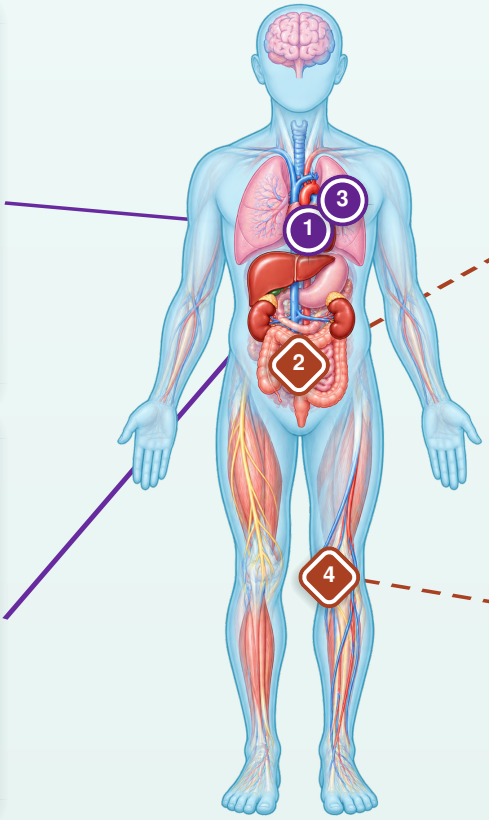
Phesgo

Keep echoes; report breathlessness, swelling or reduced exercise tolerance.

2 Diarrhoea

Especially pertuzumab

Report watery stools early, particularly with poor intake or dizziness.



3 Injection reaction

Phesgo

Tell staff immediately about rash, chest tightness or breathing difficulty.

4 Thigh injection site

Phesgo

Report persistent or spreading redness, swelling or pain.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

The body map follows the medicines active in this phase.

> Fever, shivers or suddenly unwell: contact oncology immediately. Severe chest pain, breathing difficulty, collapse or seizure: 112 / 999.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Phesgo maintenance
National Cancer Control Programme (NCCP) 00785

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **National Cancer Control Programme (NCCP) 00785 v3 · official regimen**
   https://healthservice.hse.ie/documents/6659/785_v3_Pertuzumab_trastuzumab_Phesgo.pdf
2. **ESMO 2024 early breast cancer guideline · treatment and follow-up**
   https://www.annalsofoncology.org/article/S0923-7534(23)05104-9/fulltext
3. **ESC 2022 cardio-oncology guideline · cardiac surveillance and risk**
   https://academic.oup.com/eurheartj/article/43/41/4229/6673995
4. **NICE NG101 · bone health and follow-up**
   https://www.nice.org.uk/guidance/ng101/chapter/Recommendations
5. **ESMO metastatic breast cancer guideline · 2026**
   https://www.annalsofoncology.org/article/S0923-7534(26)00902-6/fulltext
6. **EMA Phesgo product information**
   https://www.ema.europa.eu/en/documents/product-information/phesgo-epar-product-information_en.pdf
7. **ASCO · biomarkers in metastatic breast cancer**
   https://ascopubs.org/doi/10.1200/JCO.2015.61.1459
8. **HSE · chemotherapy side effects and urgent advice**
   https://www2.hse.ie/conditions/chemotherapy/side-effects/
9. **NOGG · section 7, aromatase inhibitors and fracture risk**
   https://www.nogg.org.uk/full-guideline/section-7-strategies-management-osteoporosis-and-fracture-risk
10. **eviQ · implanted port patient information**
    https://www.eviq.org.au/p/507
11. **eviQ · peripherally inserted central catheter (PICC) patient information**
    https://www.eviq.org.au/p/543
12. **NCCP NCIS antiemetics v8 (2025), Table 1**
    https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf